

Steroid-Responsive Meningitis-Arteritis (SRMA)

What is SRMA?

The **meninges** are protective layers around the brain and spinal cord. They contain blood vessels and spinal fluid.

Meningitis means white blood cells have gathered in these layers — either to fight an infection or because the **immune system attacks the body by mistake**.

SRMA is a common immune-mediated meningitis in young dogs (typically 6–18 months), especially Beagles, Boxers, Bernese Mountain Dogs and Retrievers. We know it is immune-related because levels of an antibody (IgA) are high in blood and spinal fluid, and dogs improve quickly with steroids.

Signs

- **Severe neck pain** — stiff neck, head held low, reluctance to move, crying out
- **Fever**
- Lethargy, poor appetite
- Signs often come and go

Diagnosis

- **Spinal tap** — spinal fluid shows high white blood cells (neutrophils)
- Tests to make sure there is **no infection**
- MRI or X-rays to rule out other problems

- **C-reactive protein (CRP)** blood test — supports the diagnosis and tracks response to treatment

Treatment

- **Prednisone** (a steroid) to calm the immune system
- The dose is **slowly reduced** over several months once signs improve
- We watch for steroid side effects (see our *Corticosteroids* handout)
- If signs return, another immunosuppressive drug (such as **cyclosporine**) may be added
- Rechecks with exams and CRP (and sometimes repeat spinal taps) guide the taper

Never stop prednisone suddenly — follow the tapering schedule exactly.

What to expect

The outlook is **very good**:

- About **98%** of dogs go into remission with treatment
- About half are completely cured
- **16–34%** relapse at some point, but relapses usually respond to treatment again

Call us if

Neck pain, fever or lethargy return — especially as the dose is lowered.